1 Human Rights and Coercion in Mental Health Care: Concepts, Data and Terminology

1.1 Introduction

In the following chapters, I will try to explain why coercion can no longer be justified in mental health care, including mental health nursing, and what a psychiatry without coercion could look like. I hate to admit it, but during my practical time in mental health nursing in the 1980s, I too worked on closed wards and took part in involuntary admissions as well as in more extensive physical coercive measures such as mechanical restraint and forced medication. Being involved in these measures was the most unpleasant part of the job—I hated it. It was not primarily the risk of being physically harmed during a coercive measure that made it unpleasant. Rather, it was against my personal and professional convictions.

From today's perspective, it is time to take a critical look at my own involvement in coercive measures. It is also time, and this is not just my personal opinion [1], to apologise for my own involvement to the people who had to suffer under the measures taken against their will. Of course, back in the days, I would have had virtually no opportunity to work against the system of coercive measures. In the mental health nursing profession, you were—and still are—part of a professional hierarchy and part of a chain of medical orders.

Additionally, I was able to justify it to myself, as I was dealing with people with mental illnesses who were in danger to themselves or others. Moreover, the measures were ultimately for the benefit of the patients, or so I thought. At the time, hardly anyone doubted this practice. It was a necessary evil if you worked in psychiatric hospitals. Moreover, as was also noted at the time, there was virtually no empirical research on these problems until well into the 1990s [2], on the basis of which the practice of psychiatric coercion could have been questioned.

A few publications, for example, by the psychiatrist and psychoanalyst Thomas Szasz, were of a more fundamental nature and, in addition to the existence of mental illness, doubted state intervention from a right-wing libertarian position [3, 4]. Szasz was one of the most prominent, influential and controversial researchers who

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considered the use of coercion in psychiatry to be illegitimate. His reasoning was quite simple: "Mental illnesses" are not real illnesses, as they do not—like physical illnesses—exhibit organic damage. This also removed the legitimisation of coercive measures against people diagnosed as "mentally ill".

As the German philosopher Thomas Schramme [5] noted, Szasz certainly asked the right questions—and many of the questions he posed (Can coercion be legitimised? What is actually a "mental illness"?) will crop up again in this book. However, Szasz was also a man of his scientific times, and, as of today, many of his theories would be regarded as empirically unfounded. This includes, for example, the strict separation of physical and mental phenomena, which is now considered disproved. Mental phenomena are inconceivable without physical processes; today, we have more of a problem as to whether we can still speak of a human psyche at all—but more so in the course of the book.

In the meantime, empirical research in the neurosciences and psychology has investigated many issues that are relevant to the question of the legitimisation of coercion and has produced results that allow the question of legitimisation to be answered differently today; hence, the emphasis on the fact that coercive measures are *no longer* justifiable. There are numerous studies and other results of basic research in psychiatry, against the background of which at least considerable doubts arise about the use of coercion in psychiatry. I discuss these results in detail in Chaps. 4 and 5. In my view, however, the research results go beyond the doubts. They delegitimise psychiatric measures against the will of the persons concerned. To name just two central facts, we cannot validly define mental illness and differentiate it from healthy conditions, and it is clear that the measures are predominantly not for the benefit of the people affected by them. Essentially, my aim is to show that psychiatry cannot fulfil the ethical and medical criteria it has set itself for the justification of coercion. To emphasise this clearly, the same criteria that I am criticising here as inadequate for the use of psychiatric coercion are of course also lacking in the practice of mental health nursing. Nursing professionals must also face the criticism that they do not sufficiently observe ethical standards. They must consider what consequences this should have for their own actions.

Psychiatric coercion is not just a matter of the recent past but also a connection that has been known for centuries. The involuntary segregation of people with certain behaviours that were not tolerated by other people has been handed down in Europe since the Middle Ages. As the sociologist and historian Andrew Scull reports in his cultural history of "madness", there were individual institutions that detained such people from as early as the thirteenth or fourteenth centuries [6]. However, the massive expansion of such institutions did not take place until many centuries later, as will be described in more detail in Chap. 3.

Coercion in psychiatric care is still a fundamental problem. As shown in the course of this chapter, there is evidence from various countries that involuntary placements in psychiatric institutions have increased in recent years. However, the use of coercion in psychiatric care has been subject to massive and increasing criticism, particularly in the last 20 years. The background of this is the United Nations Convention on the Rights of Persons with Disabilities of 2006 (hereinafter UN

CRPD) [7]. The Convention views the use of measures against the will of people with physical and mental disabilities from a human rights perspective and generally categorises these measures as violations of human rights. Although individual passages of the Convention, which has been ratified by almost all countries in the world, are not entirely clear and are also the subject of controversial debate in some areas [8], the objective is clear: ultimately, it is about abolishing coercion in the care of people with disabilities, which in the Convention also includes people who are considered to be mentally disordered or ill from a medical perspective.

The publication of the UN CRPD and various other documents from the human rights movement in the United Nations triggered a massive debate about whether and how measures against people's will can be justified psychiatrically. There has been considerable resistance to the Convention and its overall thrust, particularly from international medical professional organisations. This resistance intensified when the United Nations Special Rapporteur categorised coercive psychiatric measures as torture [9]. Both clinical and, interestingly, human rights arguments were put forward against the goal of abolishing coercion. From a clinical perspective, "mental disorders" are associated with a risk of impaired judgement, which could justify measures against the person's will. In terms of human rights, those in favour of psychiatric coercion argued that it could also protect, for example, the right to health or the right to life [10].

1.2 Psychiatric Coercion: What Exactly Is it?

Basically, psychiatric coercion is quite easy to define: it is any measure taken against a person's will that is necessarily justified by the existence of a diagnosed mental disorder, a mental "weakness" or an intellectual disability. As is well-known, a distinction is usually made between necessary and sufficient conditions. Such a distinction is important, in particular, regarding formal legal justifications. Sufficient legitimisation generally also requires potential danger or risk to oneself or others that has already occurred or is highly likely to occur. The justifications vary considerably from country to country.

When I wrote at the beginning of the previous paragraph that the definition is basically simple, this implies that on closer inspection, it is actually quite difficult. There is still no generally recognised taxonomy of psychiatric coercion, i.e. a classification scheme that helps distinguish between different aspects of compulsion.

In the context of psychiatric coercion, formal and informal measures are usually assumed [11]. *Formal coercion* implies a legal decision on the basis of a medical assessment. Depending on the jurisdiction and the facts of the case, there may be a court hearing and a legal decision, but often, a doctor's order is already legally binding. On the basis of these decisions, a person can be admitted to a hospital against their will, and under certain circumstances, they can also be subjected to further coercive measures, such as restraint with belts, seclusion/isolation in a separate room or an injection against their will. The legal requirements for this vary from country to country and are sometimes also inconsistent within a country, for

example, in federal structures in the United States, Germany or Switzerland. Placement in a forensic facility must also be categorised as formal coercion.

Informal coercion, also known as "soft coercion" [12], does not, as the name suggests, involve formal medical or legal measures. Here, communicative pressure is usually exerted on a person by professionals or legal representatives to achieve a certain behaviour [13]. Many formally voluntary stays in hospitals are based on such informal coercion. This ranges from a reference to enforcing a corresponding order in court if admission to the hospital is not voluntary to threats of physical violence, which—even if formally inadmissible—occur in psychiatric institutions [14]. The reference to possible formal coercion and the fact that psychiatric staff do in fact have such powers interweaves formal and informal coercion; and this is one of the issues that is difficult to distinguish.

In addition, there is another form of coercion in the mental health care system that has hardly been recognised as such and—as far as I have researched—has never been described in this way. This is *social or administrative coercion*, which the area of psychiatric rehabilitation, for example, is confronted with on an almost daily basis. Social coercion is used when certain decisions are made for people from an administrative perspective that forces them into certain life situations. This happens, for example, when legal representatives or insurance companies prevent people from utilising certain support services. One of the consequences of this is that the people affected have to work in workshops for people with disabilities instead of trying to gain a foothold in the primary labour market. Similarly, instead of living independently in their own home, they may have to live in a sheltered housing community.

The latter facts correspond to the distinction between negative and positive freedom introduced by the philosopher Isaiah Berlin [15]. "Negative freedom" refers to the absence of influence on one's personal life. "Positive freedom" refers to the possibility of developing and unfolding one's own life. While negative freedom can be associated with the absence of formal coercion, positive freedom is lost when administrative decisions restrict development opportunities.

The forms of coercion just described can be found in all countries of the world. A special form of psychiatric coercion can also be found in many low- and middle-income countries (LMICs). This refers to the segregation of people ordered or carried out by families or village elders, who are kept in cages or chained [16, 17]. These practices, which are based on long traditions, are particularly common in rural areas where psychiatric care is not available and where there is a pronounced stigmatisation of "unusual" behaviour. These practices are reminiscent of the practices known from European antiquity, in which the family had the task of keeping people with conspicuous behaviour away from the community [18].

Table 1.1 summarises the circumstances of the various types of coercion.

Another factor in this context is the subjective experience of coercion. Whether and to what extent the behaviour of employees is experienced as coercion depends not least on the context of the experience. Informal coercion can also be experienced if no measure is threatened, but the person concerned can expect this to be the case [19]. I have already questioned elsewhere [20] whether the distinction between

Table 1.1 Forms of coercion in psychiatric care

Forced type	Medium	Result	Examples
Formal	Medical-legal decision	Loss of freedom and autonomy	Forced hospitalisation, restraint, seclusion/isolation, compulsory treatment, placement in a forensic facility
Informal	Threatening and pressurising communication	Loss of autonomy	Formal voluntary but enforced intake of medication with the indication that otherwise a formal coercive measure could be taken
Social/ administrative	Medical-administrative decision	Deprivation of social development possibilities and opportunities	Decision by an authority or a legal representative in favour of a specific housing situation against the will of the person concerned
Traditional/ family	Decision of the family or a community	Isolation from social contacts	Restraining affected persons using chains or cages

positive and negative freedom is sufficient for a taxonomy, as proposed in the literature [21]. The last word has not yet been spoken about a taxonomy of coercion.

1.3 The Extent of Coercion Against People with Psychosocial Problems in the Present Day

How common is coercion in psychiatric care? Formal psychiatric coercion is assumed to affect several million people worldwide every year. A recent analysis of published data on coercive measures from countries in the Western world has shown that a median of 120 out of 100,000 people are affected by formal measures each year [22]. Extrapolated to the population of the 27 EU member states, this would already affect more than half a million people per year in this relatively small part of the world. However, there is a lack of relevant data and up-to-date statistics in many countries, which, in my view, is a scandal.

A comparative study of the incidence of involuntary hospitalisations in Europe, Australia and New Zealand revealed that these admissions increased significantly in most countries between 2008 and 2017 [23]. As an update, I undertook my own nonsystematic literature search of time series on involuntary measures in mental health care after this period. This revealed the following picture. Increases in involuntary measures have been reported in Brazil [24], Italy [25], Croatia [26], Switzerland [27], Canada [28–30], Australia [31], England and Wales [32–34], Scotland [35], Germany [36, 37] and the Netherlands [38]. Different developments have been reported in the United States. The majority of states reported an increase in coercive measures, whereas a minority of states reported a decrease [39].

Relatively unchanged figures were reported from China [40] and Finland [41]. In New Zealand, there was no significant change across the board, but there were differences in various forms of restraint, namely, a decrease in seclusion/isolation but an increase in community treatment orders [42]. Significant decreases in coercive measures were reported in Ireland [43] and Oregon, USA [44].

This means that there has been an overall increase in psychiatric coercion in many countries in recent decades, at least according to published data. However, these data are fraught with considerable methodological problems, particularly with regard to international comparability. The relevant factors here include different legal frameworks, different data collection procedures and problems with data quality [45]. In addition, these data are generally not transferable to low- and middle-income countries [22].

In my view, the expansion of formal coercion beyond the already extensive forms is worrying. I am not referring to the return of politically punitive psychiatry in Russia [46] on the basis of what is described as "archaic" care [47] or the abuse of psychiatry in Iran [48]. Rather, it refers to political intentions in the United States to consider homelessness alone as an indication of compulsory hospitalisation [49]. This also refers to the extension of coercion to the outpatient sector, where community treatment orders are increasingly seen as a "humane" alternative to placement in a psychiatric hospital [50].

This expansion can also be seen in another sector, namely, forensic hospitalisation. For several decades, the number of forensic psychiatric beds in the Western world has increased significantly [51], and this has also been the case in Germany, for example, until recently [52]. A general methodological difficulty with these data is the inconsistent collection from different sectors of care. For example, forensic hospitalisation is not usually included in health care systems research. A corresponding data analysis from various European countries has shown that a comparatively large number of people are forensically detained in the Netherlands and Belgium, whereas relatively few people are in Spain and Italy [53]. In addition to aspects of care systems, the extent to which different legal traditions prevail in different countries, particularly in the area of forensics, must also be considered [54].

Another area that goes hand in hand with psychiatric coercion in various respects is a distinct black box from a scientific perspective. This refers to the instrument of guardianship. Even special data collection by specialists in this field revealed that there are no figures on the extent for most countries, even in the Western world, where the data availability is usually better [55]. One particular difficulty lies in the major differences in the exercise of legal representation. As part of a reform of German law, for example, the concept of the best interests of the person was replaced in favour of the will of the person [56] to take into account the concerns arising from the UN CRPD. Guardians are now obliged to follow the wishes of the person being cared for more closely. However, this is unlikely to have any effect on the practice of coercive measures, as first reviews are suggesting [57]. The percentage of people under guardianship remains particularly high in Germany. According to a publication by the German government, 1.5 percent of the adult population was under legal guardianship in 2015 [58]. It is particularly worrying that 15 percent of these people

are unable to exercise their rights at all (German: *Einwilligungsvorbehalt*). For a large proportion of these people, this is due to a psychiatric diagnosis.

Many of these people live in a "special form of accommodation", as inpatient facilities (care homes, outpatient residential group settings, etc.) are called today. In Germany, there were a total of 194,000 people in 2020, 55,800 of whom were people with a "mental disability" ([59]; own calculations). The number of places in residential homes has been increasing internationally for a long period of time [51, 60]. What has rarely been investigated internationally, however, is the number of people who are placed in such residential settings against their will. It is also unclear how many closed residential care places are actually provided for people with psychosocial problems. In Germany, approximately 10 percent of all places in a psychiatric residential home are closed [61], and there is a call—particularly from a professional perspective—for significantly more closed places.

According to the results of a comparative study of living conditions in Germany, slightly less than 50 percent of the respondents in the "special form of housing" who took part in this study were unable to decide wholly on their own which form of housing they wanted to live in. However, even in the outpatient setting, not all respondents believed that they could decide their living arrangements independently. Here, the proportion was still 21 percent [62]. These data make it clear that social-administrative coercion is exercised in areas that cannot necessarily be regarded as formal coercion. It is often the authorities or funding organisations that prevent the social situation of affected people from developing further.

This fact also becomes clear when housing preferences are considered. Housing preferences have been relatively well researched. A corresponding meta-analysis that I conducted revealed that more than 80 percent of the people surveyed with a psychosocial problem wanted to live in their own home [63]. Another meta-analysis in which I was involved showed that more than 60 percent of the people surveyed wanted to be employed in the general labour market and not in workshops or day centres [64]. This also raises the question of how many people are denied access to the general labour market against their will. For Western countries such as Germany, it is estimated that less than one percent of eligible people actually have access to evidence-based programmes such as Supported Employment, which are able to make a place in the primary labour market achievable [65].

In summary, it should be noted that psychiatric coercion—unlike political initiatives suggest and affected individuals and professionals aim for—is not decreasing in most countries but rather is increasing. Furthermore, it should have become clear in which hitherto little-known sector of psychiatric care social and administrative coercion is exercised and that this takes place to a considerable extent, even if exact figures are not known.

1.4 Towards a Nondiscriminatory and Non-pejorative Terminology

Throughout this book, I use terms that may be considered unusual in the context of psychiatry and mental health nursing. Instead of "mental illness" and "disability", I refer to "psychosocial problems" unless otherwise stated and justified, and instead of "psychiatric care", I prefer "psychosocial support".

Why do I think this is necessary? In the following chapters, I will explain why coercion in psychiatric care can no longer be justified and what the consequences of "psychiatry without coercion" would be. However, I am convinced that "psychiatry without coercion" can hardly be realised if we have not questioned and, in some way, overcome our conventional concepts of mental illness. Coercion in psychiatric care is based, as will be explained in detail, on the conviction that a psychiatric diagnosis in combination with risk behaviour legitimises measures against a person's will. A diagnosed mental disorder or a comparable condition is therefore a necessary component of legitimisation. However, and this will also be discussed in detail, the existence of a disorder, as it is usually diagnosed today, is increasingly uncertain, especially from a neuroscientific perspective. This means that the definition and identification of a mental disorder are currently increasingly unclear. This also blurs the boundaries between different psychiatric diagnoses and, above all, between the states of disordered/non-disordered or ill/non-ill.

At the same time, I want to express that we need a terminology in psychiatry and mental health nursing that is more inclusive and diverse in the sense that people who may not define themselves as "ill" are taken into account, even if they have support needs that may not be "psychiatric" in the strictest sense. This could be support from peers, i.e. people who are also affected by psychosocial problems. Up to 80 percent of people who have been diagnosed with "schizophrenia" have no corresponding "insight" [66]. I would like to prevent something that is described in medical ethics and epistemology as epistemic injustice [67]. Epistemic injustice can arise when people do not feel that their specific views and knowledge are adequately recognised, for example, or when people with psychosocial problems are labelled "disabled" or "ill" against their will. Other areas where epistemic injustices have been identified include gender issues or ethnic minorities, who have been discriminated against linguistically and otherwise.

Language, and written language in particular, can be politically charged, as we have learned in recent years in connection with the discussion about gendered language. Even if women were "included" in male terms in certain languages, this was not enough for many women to feel adequately represented. The same applies to terms related to illness, disability or impairment. Just as purely masculine language can be perceived as sexist, terminology that refers to psychosocial problems as "disabled" is in danger of encouraging "ableism", as already mentioned. *Ableism* refers to discrimination against people who cannot fulfil certain ability norms [68].

Even longer than *ableism*, the term *sanism* has been used to describe discrimination in connection with (mental) illness [69]. The starting point for this analysis in the early 1990s was stereotypes used in legal proceedings and articles, for example,

that people with psychosocial problems were "weak", "lazy", "sexually uncontrollable", "dangerous" and fundamentally unable to manage their affairs autonomously.

In scientific texts or specialist books, we are also constantly exposed to the danger of using *dismissive* or *sanitised language* [70]. It has long been known from psychiatric stigma research, for example, what consequences this can have for the people affected. According to the sociologist Erving Goffman, in his seminal work on the subject, a person to whom we ascribe a stigma is not regarded as a person of equal value [71]. As Goffman described in detail in the 1960s, this has a massive impact on life and social identity. Therefore, when we speak of "mentally ill people" instead of "people with psychosocial problems", we reduce the identity of these people by making their humanity disappear, and we also do not care about the question of whether the people see themselves as ill. In this respect, we devalue and assign a certain characteristic to the person concerned, which can be humiliating. However, this can also be seen quite differently by the people concerned, as will become clear in a moment.

Of course, as Goffman also described at the time, we generally do not use these terms with the intention of stigmatising. In addition, in medical or nursing professions in particular, it is actually our idea that we want to be supportive with our actions and language. Nevertheless, we have to deal with perspectives that may experience this differently. In addition, for some years now, we have had to deal with activists who expect us professionals to use the language more carefully. In this respect, it is an advantage if the people concerned communicate clearly which terms are more appropriate and which are not. The first guidelines on how researchers can avoid using ableist language have already been published [72], and I try to follow them as closely as possible.

However, such orientations are neither easy to implement nor uncontroversial. We live in sociodiversity, as explained in Chap. 6, and in such a society, there are very different and sometimes contradictory experiences and attitudes towards the terms commonly used in the psychiatric field. I referred above to the now common term "people with… problems". This terminology, which is referred to as *person-first language*, was assumed to be less stigmatising, as it focuses on the person and not the problem [73]. However, this does not necessarily have to be the case, as is communicated mainly by the autism movement. There, an *identity-first language* is favoured in the sense of the neurodiversity terminology [74]. Instead of "people with autism", "autistic people" is expected. The reasoning: Autism is not a deficit that should be less stigmatised but rather an identity with positive connotations, something autistic people can be proud of. Like *identity first* instead of *person first*, terms such as "comorbid" should be replaced by "co-occurring", "symptoms" by "experiences" and "treatment" by "support" [75].

As one can expect, there is resistance to this obvious de-medicalisation of language and normalisation of the phenomena, particularly from parents' associations of autistic people. The argument: Since autism spectrum disorder is a defined clinical condition in the classifications of psychiatry, it must be possible to speak "scientifically accurately" about these disorders and, above all, about the massive limitations, disabilities and deficits of the people affected [76]. As much as I can

understand this demand from the parents' point of view, it is not very helpful for the issue under discussion here. From a scientific and epistemological perspective, however, which aims to discuss the significance of "illness" in connection with coercion in psychiatry, it is expedient to *avoid ableism* and *sanism* as much as possible.

At the beginning of this book, as already mentioned, basic terminological definitions should therefore be made. To ensure that a terminology that enables inclusion and diversity is used without prior discrimination or devaluation, the term "psychological phenomenon" should first be defined. A psychological phenomenon describes what every person experiences of themselves at any given time, for example, being in a bad mood or, on the contrary, being completely happy. However, such a state can also mean hearing voices or a strong desire to consume drugs. Importantly, in this context, no judgement is associated with this state. A phenomenon is not necessarily a problem.

However, a psychological condition can result in a "psychosocial problem". The "psychosocial problem" is the second term that I would like to introduce here. A psychosocial problem can arise when the person in question experiences negative consequences from the condition, such as suffering. However, it can also be—and this is why we are not just talking about psychological but psychosocial consequences—a social consequence associated with the phenomenon, for example, the separation of a partner or the loss of a job. Finally, other people can be affected in a social sense, for example, through aggressive behaviour or threats to children's welfare. When psychosocial support systems address psychological phenomena, they are, by definition, problems that have been recognised somewhere and have led to support systems being called in.

I realise that the term "problem" can in some ways have negative connotations that can be interpreted in terms of *ableism* or *sanism*. Not everyone involved necessarily has to subscribe to the view of the problem; as we know, this is how many conflicts arise in psychiatric settings. As I said, my aim is to avoid these connotations as much as possible, which will not always be possible. The difficulty is inherent because support systems such as psychiatric services currently have the power to define what is a problem and what is not. Bearing these difficulties in mind, I use the term "psychosocial problem" as an extension or umbrella term for "mental disorder" or "illness". In Chap. 6, all the facts of the phenomena and problems are described in more detail.

The third term I would like to introduce is "psychosocial support". This is intended to describe any form that people with a psychosocial problem experience as a potential help or solution. As an extension of "psychiatric care", this also includes nonmedical support such as coaching (e.g. job coaching) or peer support from people who are also affected by psychosocial problems. However, conventional treatment and therapy procedures can also be categorised as psychosocial support.

These terms should be used as much as possible. Conventional terminology such as disease, disorder or care will be applied to existing situations and, if they are critically scrutinised, placed in inverted commas.

1.5 The Plan of the Book

The first part of this book aims to show that psychiatric coercion can no longer be justified. At the beginning, Chap. 2 identifies the legitimisation problems for psychiatry. This is followed in Chap. 3 by a reconstruction of the current justification of coercion in psychiatry. Chapters 4 and 5 use research literature to justify that the ethical and legal conditions for the use of coercion do not hold up in light of research.

The second part will then show ways of overcoming coercion in psychiatry. My assumption is that this can be achieved only through fundamental changes to the concept of illness in psychiatry; this is described in Chap. 6. In addition, relevant changes in the support of people with psychosocial problems will be necessary to overcome coercion, as analysed in Chap. 7. Chapter 8 highlights the possible consequences of not using psychiatric interventions against people's will, which can lead to numerous dilemmas. In the concluding epilogue in Chap. 9, I will venture an outlook on further developments, which—this much I can already reveal—will not be particularly optimistic.

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